

# HOW TO OPEN A BRAIN INJURY ADULT RESIDENTIAL FACILITY in California

A CDSS-licensed Adult Residential Facility (ages 18–59) — funded mainly through SSI/SSP.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

## The California model — read this first

In California, a home that provides 24-hour non-medical care and supervision to working-age adults with brain injury is licensed by the California Department of Social Services (CDSS), Community Care Licensing Division (CCLD), Adult and Senior Care Program, as an Adult Residential Facility (ARF) under Title 22, Division 6, Chapter 6 and the California Community Care Facilities Act (Health & Safety Code § 1500 et seq.). An ARF serves adults ages 18–59 with disabilities who need care and supervision — so a working-age brain-injury home is an ARF. This guide walks the whole path — licensing the home, then getting paid — broken down so a first-timer can follow it.

***The single most important California fact — read before you build a business plan.*** California funds residential services for people with DEVELOPMENTAL disabilities through its 21 Regional Centers — but a developmental disability must begin BEFORE age 18. An adult who acquired a brain injury later (adult-onset TBI) generally does NOT qualify for Regional Center funding. That changes everything about your money: your payers are mainly SSI/SSP board-and-care (a fixed monthly rate) and private pay — not a Medicaid per-diem. It also limits secured/locked settings (see Part 4). Build your revenue model around SSI/SSP + private pay from day one (Part 8).

***ARF vs. RCFE — pick by age.*** An ARF serves adults 18–59. A Residential Care Facility for the Elderly (RCFE) serves people 60 and older, under a different chapter with a longer (80-hour) administrator course. A working-age brain-injury home is an ARF; if you ever serve someone 60+, that person belongs in an RCFE. Don't mix the licenses.

***Accreditation — strongly recommended, and how we build.*** California does not require national accreditation to hold an ARF license, but for a brain-injury program it is a real advantage: CARF (which offers a Brain Injury specialty) and The Joint Commission signal quality to hospital discharge planners (including Rancho Los Amigos), case managers, and families. Every PolicyReady product is built to those standards behind the scenes. Treat accreditation as a goal from day one (see Parts 7 & 9).

## Before you begin — a realistic picture

Be clear-eyed about what you're starting: this is a licensed, capital-and-staffing business, not a side project. You are asking the State of California to trust you with vulnerable adults in a home that runs 24 hours a day, every day, so plan for a real approval timeline (CDSS targets 90–120 days from a COMPLETE application, and the fire clearance, background checks, and any construction routinely push that out) and a working-capital cushion that carries payroll, rent, food, and insurance through the months before your beds fill. CCLD will not license you until you show at least three months of operating reserve in the applicant's name — treat that as a floor, not a target. The two facts that most shape the economics are covered in the callouts above: your main payer is the FIXED SSI/SSP board-and-care rate (about \$1,626/month per resident in 2026), and you generally cannot lock the perimeter for adult-onset brain-injury residents. Both mean a brain-injury ARF succeeds by blending SSI/SSP residents with private-pay families and by staffing to acuity — so build your model on those realities before you sign a lease.

## How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It is written for someone who has never opened a facility — where we name a thing (an agency, a form, a rule, an insurance type), we break it down and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; verify each step with CCLD, your fire authority, and qualified counsel (see Part 11) before you rely on it.

## The journey at a glance

Ten milestones take you from idea to a licensed, paid home. Each part below is one rung.

|    |                                       |                                                                                    |
|----|---------------------------------------|------------------------------------------------------------------------------------|
| 1  | <b>Decide this is your business</b>   | A licensed, 24-hour ARF for brain-injury adults 18–59 (Part 1)                     |
| 2  | <b>Form the business &amp; insure</b> | Entity, EIN, bank, and the right insurance — broken all the way down (Part 3)      |
| 3  | <b>Secure &amp; prepare the home</b>  | Zoning, and the Title 22 physical-plant specs (Parts 3–4)                          |
| 4  | <b>Certify your administrator</b>     | The 35-hour ARF ICTP + the state exam (Part 5)                                     |
| 5  | <b>Build &amp; train staff</b>        | Live Scan clearances, first aid/CPR, medication assistance (Part 5)                |
| 6  | <b>Write your Plan of Operation</b>   | Plan of Operation, P&P, and the required forms (Parts 4–6)                         |
| 7  | <b>Get the CDSS license</b>           | Orientation → LIC 200 → fire clearance → pre-licensing visit (Part 6)              |
| 8  | <b>Set up your funding</b>            | SSI/SSP board & care + private pay (Regional Center only if DD before 18) (Part 8) |
| 9  | <b>Get your first residents</b>       | Rancho / hospital discharge planners + county behavioral health (Part 10)          |
| 10 | <b>Who to call &amp; what to ask</b>  | Every contact + the exact questions, all in one place (Part 11)                    |

**Rule of thumb.** Attend the CCLD orientation and start your administrator's 35-hour course and everyone's Live Scan early — the orientation is required BEFORE you can even apply. Do the cheap, reversible steps (form the entity, sign a contingent lease) first, and budget the \$800 California franchise tax and three months of operating reserve.

## PART 1

## What This Business Is

**In short:** A CDSS-licensed Adult Residential Facility — 24-hour NON-MEDICAL care and supervision for adults 18–59 with brain injury. It's a real-estate + 24-hour-care business, paid mostly by SSI/SSP and private pay.

California law defines an Adult Residential Facility as a community care facility that provides 24-hour non-medical care and supervision to adults ages 18–59 with physical, developmental, and/or mental disabilities who need care and supervision. “Non-medical” is the key word: an ARF is not a nursing facility and provides no skilled nursing — it provides help with daily living, supervision, and assistance with self-administered medication. It's licensed for the level of care and supervision your residents need, so a home built for adults with brain injury operates as an ARF.

### ARF capacity categories

| Capacity              | Notes                                                                                                  |
|-----------------------|--------------------------------------------------------------------------------------------------------|
| 1–6 residents (small) | Treated as a residential use for zoning — must be allowed in residential zones (HSC §§ 1566.3, 1267.8) |
| 7–15 residents        | A larger license tier                                                                                  |
| 16–30, 31–60, 61+     | Progressively larger tiers with higher fees and staffing expectations                                  |

Most brain-injury ARFs start small (six or fewer), which also gives you the residential-zoning protection — a real advantage when siting the home.

### Serving residents who wander — a hard California limit

This is a place California is stricter than most states. Secured perimeters and delayed-egress locks in an ARF are authorized **ONLY** for residents who have a developmental disability, receive Regional Center services, are 14 or older, in facilities of six or fewer, with State Fire Marshal and Department of Developmental Services approval, and under a statewide cap of 174 beds (HSC § 1531.15). Because adult-onset brain-injury residents generally do **NOT** have Regional Center/developmental-disability status, you generally **CANNOT** run a secured-perimeter/locked ARF for them. Plan wander-risk management through supervision, environmental design, door alarms, and admission screening instead — and confirm any approach with CCLD and counsel.

**Key terms.** CDSS = California Dept. of Social Services (your licensor). CCLD = Community Care Licensing Division. ASCP = Adult and Senior Care Program. ARF = Adult Residential Facility (ages 18–59). RCFE = the 60+ version. Title 22 = the regulations. ICTP = Initial Certification Training Program (the administrator course). SSI/SSP = federal Supplemental Security Income + California's State Supplementary Payment. NMOHC = Non-Medical Out-of-Home Care (the SSI/SSP board-and-care rate). Regional Center = the DD-services system (Lanterman Act). Live Scan = electronic fingerprinting. NPI = National Provider Identifier. CARF/TJC = the two accreditors.

## PART 2

# Who Regulates You (and What Each One Does)

**In short:** *CDSS/CCLD licenses you, your local fire authority clears the building, SSI/SSP (Social Security + California) pays board & care, and the Long-Term Care/community-care oversight protects residents.*

### **CDSS / CCLD — Adult and Senior Care Program (your licensor)**

The California Department of Social Services, Community Care Licensing Division, Adult and Senior Care Program, issues your ARF license, writes the Title 22 standards, runs the required orientation, reviews your Plan of Operation, conducts the pre-licensing inspection, and inspects the facility over time. Centralized Applications Bureau (833) 827-6084; Adult Care Licensing (916) 657-2592; [cdss.ca.gov](http://cdss.ca.gov) (Adult Care Licensing).

### **The local fire authority / Office of the State Fire Marshal**

Before CDSS licenses you, your local fire authority (or the State Fire Marshal) issues the fire clearance for your occupancy. For a brain-injury home whose residents may not self-evacuate, engage them early — and note the secured-perimeter limits above.

### **Social Security / California SSP (who pays board & care)**

Most ARF residents are paid for through SSI/SSP — federal Supplemental Security Income plus California's State Supplementary Payment — at the Non-Medical Out-of-Home Care (NMOHC) rate, which is a fixed monthly board-and-care amount, not a Medicaid per-diem. The Social Security Administration determines SSI eligibility; California adds the SSP. (Regional Centers pay only for the minority of residents whose disability began before 18 — Part 8.)

### **Community-care oversight & Adult Protective Services**

CCLD investigates complaints and protects residents' rights in licensed facilities; the Long-Term Care Ombudsman and county Adult Protective Services (APS) take abuse/neglect concerns. You are a mandated reporter — report suspected abuse, neglect, or exploitation. County APS statewide line 1-833-401-0832.

## PART 3

# Form Your Business & Secure the Home — Step by Step

**In short:** *This is where people get stuck. We break each piece down: what it is, where to go, and exactly what to do.*

## 3.1 Form your legal entity

An “entity” is the legal business (usually an LLC) that holds the license and bank account and shields your personal assets. A community care facility is licensed under the Health & Safety Code (not a professional license), so an ARF may be run by an LLC or a corporation. You form it at the California Secretary of State.

- 1. Go to the California Secretary of State.** File online at [bizfileOnline.sos.ca.gov](https://bizfileOnline.sos.ca.gov) (Business Programs Division, 916-657-5448).
- 2. File Articles of Organization (Form LLC-1).** The filing fee is \$70. Name the LLC and list a registered agent (receives legal mail — can be you).
- 3. File your Statement of Information (Form LLC-12).** \$20, due within 90 days of forming, then every two years.
- 4. Budget the \$800 franchise tax.** California charges every LLC a minimum \$800 annual franchise tax (Franchise Tax Board). The old “first year free” break expired at the end of 2023, so an LLC you form now owes the \$800 for its first tax year — plan for it.

*California has no LLC newspaper-publication step. Unlike New York or Arizona, California does not require you to publish a formation notice — but the \$800 franchise tax and the biennial Statement of Information are real recurring obligations to calendar.*

## 3.2 Get your EIN (federal tax ID)

An EIN is your business's federal tax number — free from the IRS, and you need it for the bank and payroll.

- 1. Go to IRS.gov** and search “Apply for an EIN online” (the EIN Assistant). It's free — never pay a third-party site.
- 2. Apply online** with the responsible party's name and SSN/ITIN and your entity info.
- 3. Save the confirmation (CP-575)** — you get the EIN immediately.

## 3.3 Open a business bank account

**What to have ready:**

- EIN confirmation from the IRS
- Your Articles of Organization (LLC-1) and the entity number
- Operating Agreement (LLC)
- A Beneficial Ownership form (owners of 25%+ and a control person)
- Government photo ID, SSN, DOB, home address for each owner/signer
- An opening deposit (often ~\$100)

Where to go: a major bank with lots of California branches — Chase, Bank of America, Wells Fargo, US Bank, or a local credit union. You'll also need to show CCLD you have at least three months of operating reserve in an account in the applicant's name, so ask about business savings too. Compare monthly fees and how to waive them, transaction/cash-deposit limits, and whether they offer a business line of credit.

## 3.4 Get your insurance — what you need, who sells it, and what to ask

An ARF cares for vulnerable adults in a building, so you need several kinds of coverage — not one policy:

| Coverage                                   | What it protects against                                           | Must-have?                    |
|--------------------------------------------|--------------------------------------------------------------------|-------------------------------|
| General Liability (GL)                     | A visitor/family slip-and-fall or property damage on your premises | Yes — foundational            |
| Professional Liability (malpractice / E&O) | Claims about the CARE — supervision, med errors, neglect           | Yes — top healthcare coverage |

|                                         |                                                              |                                                 |
|-----------------------------------------|--------------------------------------------------------------|-------------------------------------------------|
| Abuse & Molestation                     | Allegations of abuse by staff or others (GL excludes this!)  | Yes — high exposure                             |
| Commercial Property                     | The building, contents, equipment; add Business Interruption | Yes if you own; often if you lease              |
| Commercial Auto (incl. hired/non-owned) | Transporting residents; staff driving own cars for work      | Yes if you drive residents                      |
| Workers' Compensation                   | Staff injuries (lifting/transferring is high-risk)           | Required in California from your first employee |
| Cyber Liability                         | A breach of protected health information (HIPAA)             | Strongly recommended                            |

**Workers' comp is mandatory from employee #1 in California.** California requires workers' compensation as soon as you have any employee — set up your policy before your first hire.

### Real places to get it

Buy through an independent agent/broker who accesses specialty carriers. Carriers that write residential-care/social-service risk include Philadelphia Insurance (PHLY), CNA, Markel, Berkshire Hathaway GUARD, Great American, and Nonprofits Insurance Alliance of California (NIAC, for nonprofits). Specialty programs/brokers for this niche include Caitlin Morgan, Gallagher, AssuredPartners, and XInsurance. To find a local California agent, use the “Trusted Choice” Find-an-Agent directory at [trustedchoice.com](https://www.trustedchoice.com) (filter healthcare/senior-care/social-service), or ask a licensed peer. Verify each carrier writes adult residential facilities in California before relying on a quote.

### What to ask the insurance agent

*“Do you write adult residential facilities / brain-injury homes — which carriers?”*

*“Is professional liability included — claims-made or occurrence — and what limits?”*

*“Is abuse & molestation included, at what limit, sub-limit or full?”*

*“Are GL and professional liability shared limits or separate?”*

*“What limits does my landlord/lender (or a placing agency) require?”*

*“How do I set up California workers' comp, and what will it cost given my staffing?”*

## 3.5 Secure & zone the home

- ☐ For 6 or fewer residents, California law treats the ARF as a residential use — it must be allowed in residential zones (HSC §§ 1566.3, 1267.8). Confirm with your city/county planning office and get it in writing.
- ☐ Confirm the building can meet the Title 22 physical-plant specs (Part 4) and be made accessible.
- ☐ Identify your local fire authority early and confirm the fire-clearance path for your occupancy and resident count.
- ☐ If leasing, make sure the lease allows your use and resident count, and make it contingent on licensure.

## 3.6 Map ownership + Live Scan clearances

Identify every owner and controlling person — the licensee, administrator, every employee with client contact, and any adult living in the home must clear a criminal-record check via Live Scan (Part 5.4) before licensure. A prior conviction isn't an automatic bar; there's an exemption process (HSC § 1522).

## 3.7 Get your NPI (National Provider Identifier)

An NPI is a free national ID. Even though an ARF is paid mainly through SSI/SSP rather than a health plan, get a Type 2 (Organizational) NPI — you'll want it if you ever vendorize with a Regional Center, bill any Medi-Cal service, or seek accreditation:

1. **Create an I&A account** at [nppes.cms.hhs.gov](https://nppes.cms.hhs.gov) (Identity & Access) for the person who'll manage the number (usually an owner).
2. **Log into NPPES** and start a new application; choose Type 2 (Organizational).

**3. Enter your legal name, EIN, and address** and pick the taxonomy that fits a residential/community-care facility.

**4. Submit — it's free** and usually processes in about 10 business days.

**What this means for you.** *None of Part 3 requires a license or a clinician — it's paperwork you can finish in a week or two, so do it early and in parallel with orientation and the administrator course. The order that saves the most rework: form the LLC first (you need the entity number for everything downstream), then get the EIN (you need it for the bank), then open the bank account and fund the reserve, then buy insurance and get the NPI. Keep every confirmation — the Articles, the EIN letter, the Operating Agreement, the insurance binders, the NPI — in one folder; CCLD's analyst will ask for most of them, and a bank or landlord may ask for the rest.*

**Sequencing tip — don't over-commit before zoning is confirmed.** *The one step in Part 3 that can strand money is the home. Get the residential-zoning confirmation in writing and confirm the fire-clearance path (Part 4) BEFORE you sign anything non-contingent or pay for renovations. A lease that's contingent on licensure, and a purchase offer with a licensing/zoning contingency, keep your options open if the building can't meet Title 22.*

## PART 4

# The Home — Physical Plant, Safety & Fire Clearance (Title 22)

**In short:** Title 22 sets bedroom, bathroom, and temperature rules; your building's floor area comes from the Building Code; and a fire clearance is required before licensure. Here's what each means and how it's checked.

“Physical plant” just means the building itself and everything fixed in it — the rooms, plumbing, and safety features. CCLD inspects the facility and your local fire authority issues the fire clearance before you're licensed. The rules live in Title 22, Division 6, Chapter 6 (and the general Chapter 1). Below, each requirement is followed by what it means for you.

### 4.1 Bedrooms & space (Title 22 § 85087)

- ☐ No more than 2 residents per bedroom (waivers to exceed require CCLD approval)
- ☐ Bedrooms large enough for easy passage and comfortable use of any required assistive devices (wheelchairs, walkers)
- ☐ No room commonly used for other purposes (living room, garage) may be a bedroom; a bedroom may not be a passageway to another room
- ☐ Each resident has their own bed and at least 2 drawers / 8 cubic feet of drawer storage

What this means for you: California's ARF rules cap bedrooms at TWO residents (stricter than the four many states allow), and — importantly — Title 22 does not set a per-bed square-footage number. Your minimum floor area comes from the California Building Code (commonly about 70 sq ft for the first occupant, more per added person) and is enforced through the building/fire clearance. Don't apply the RCFE 70/60-sq-ft figures to an ARF; confirm your floor area with the building department. Brain-injury residents often use wheelchairs, so plan real turning room.

### 4.2 Bathrooms, temperature & water (Title 22 §§ 85088, 80088)

- **Toilets & sinks:** at least one toilet and one washbasin for every 6 persons residing in the facility.
- **Bathing:** at least one bathtub or shower for every 10 persons.
- **Safety:** grab bars at toilets and in tubs/showers, non-slip surfaces, and features for residents using wheelchairs/walkers/transfer benches.
- **Temperature:** keep occupied areas between 68°F and 85°F.
- **Hot water:** regulate hot water at personal-care fixtures to 105–120°F; water at 125°F or above requires warning signage. Set the heater and verify with a thermometer.

What this means for you: think like a licensing analyst walking your bathrooms and kitchen. Bathroom ratios, safe water temperature, and safe storage of chemicals and medications are common early findings. Verify the water temperature, add grab bars, and lock up chemicals and medications before your pre-licensing visit.

### 4.3 Fire clearance & life safety

- ☐ A fire clearance from your local fire authority (or the State Fire Marshal) for your occupancy and resident count — required before licensure
- ☐ Smoke detectors and fire extinguishers per the fire authority; clearly marked exits
- ☐ A written emergency and disaster plan; fire drills; staff trained to move residents who can't self-evacuate
- ☐ Remember the secured-perimeter limit (Part 1) — locked/delayed-egress settings are generally not available for adult-onset TBI residents
- ☐ Keep every fire and licensing inspection report in one dated compliance binder

**Get the fire authority in early.** The fire clearance is a gating step, and for a brain-injury home the residents' evacuation ability drives what's required. Identify your local fire authority and confirm the path before you commit to or renovate a building.

#### 4.4 Brain-injury environmental design (why it matters here)

Because you generally can't lock the perimeter, design and supervision do the work: clear, low-clutter walking paths to reduce falls; consistent, labeled locations for personal items (memory and orientation support); door/exit alarms and line-of-sight layouts to manage elopement risk without a secured perimeter; reduced noise and visual overstimulation in common areas; and a quiet space a dysregulated resident can use. This is both good care and how you keep wander-risk residents safe within the ARF rules.

#### 4.5 Walk the home before the analyst does — a pre-inspection self-check

The pre-licensing visit goes far better when you've already caught the common findings yourself. Walk the home with this list a week before, fix what you find, and photograph it for your compliance binder:

- ☐ Bedrooms: no more than 2 residents each; each resident has a bed and at least 2 drawers / 8 cubic feet of storage; no bedroom doubles as a passageway or a converted living room/garage
- ☐ Bathrooms: at least one toilet and washbasin per 6 residents and one tub/shower per 10; grab bars installed at toilets and tubs/showers; non-slip surfaces down
- ☐ Water & temperature: hot water at personal-care fixtures reads 105–120°F on a thermometer (warning signage if 125°F+); occupied rooms hold 68–85°F
- ☐ Chemicals and medications locked and stored separately from food; cleaning supplies out of resident reach
- ☐ Fire & exits: smoke detectors and extinguishers per the fire authority; exits clearly marked and unobstructed; the written emergency/disaster plan posted and drills logged
- ☐ Food: a 7-day supply of non-perishable food on hand for the inspection
- ☐ Wander safety: door/exit alarms working; line-of-sight layout in common areas; a quiet, low-stimulation space available

**What this means for you.** Nearly every early deficiency on an ARF report is something on this list — water temperature, missing grab bars, unlocked chemicals or medications, or bathroom ratios. None is expensive to fix; all are cheap to miss. A clean pre-licensing visit shaves weeks off your timeline, so treat this self-check as the last thing you do before you call CCLD to schedule.

## PART 5

## Staff, the Administrator & Required Training — by Position

**In short:** California is specific about the administrator's certification and background checks; on direct-care staff it sets required topics more than a fixed hour count. Here it is, one by one.

### 5.1 Certify your administrator (Title 22 §§ 85064, 85096)

Every ARF must have a certified administrator (who may be you). Here's exactly how to get certified:

- 1. Complete the ARF Initial Certification Training Program (ICTP).** 35 classroom hours from a CDSS-approved vendor, covering laws & residents' rights, management, psychosocial and physical needs, medication management, admission/assessment, community resources, and crisis intervention. (Note: this is the ARF course — the RCFE course is a longer 80 hours.)
- 2. Pass the state exam.** Take the state-administered written exam within 60 days of finishing the course — up to 3 attempts, and the \$100 exam fee covers all three.
- 3. Apply for the certificate (Form LIC 9214).** Within 30 days of passing, submit your application with your criminal-record statement and Live Scan, proof of training and exam, and the \$100 processing fee.
- 4. Recertify every 2 years.** 40 hours of continuing education per 2-year period, including at least 4 hours on ARF laws/regulations. Up to half the CE may be interactive online; renew within 90 days before expiry (a late renewal within 4 years adds a \$300 fee; after 4 years you must start over).

*Start this early.* The 35-hour course plus the exam take real time to schedule and finish, and you can't be licensed without a certified administrator. Book a CDSS-approved ICTP before you apply.

### 5.2 Direct-care staff — required topics (Title 22 § 85065)

California requires you to “employ staff as necessary to ensure provision of care and supervision to meet client needs,” and to train direct-care staff on a set of TOPICS rather than a single fixed clock-hour count. Train (and document) staff on:

- **The required topics:** resident care and communication; nutrition and safe food handling; housekeeping and sanitation; assisting with self-administered medication; recognizing the early signs of illness and when to get help; residents' rights; and community resources.
- **First aid / CPR:** have staff trained in first aid and CPR available; confirm the current specific requirement with CCLD, since the numeric mandate isn't fixed in the ARF regulations the way it is for RCFEs.
- **Brain-injury competence (build it in):** California doesn't mandate a TBI module, so add your own — cognitive stimulation, wander- and personal-safety without a secured perimeter, de-escalating dysregulated behavior, and communication strategies for cognitive/speech impairment.

*Don't copy RCFE hour figures.* You'll see “40 hours initial / 20 hours annual” online for California care staff — that's the RCFE elderly-facility rule (HSC § 1562.3), not the ARF rule. Confirm the current ARF staff-training requirements directly with CCLD rather than assuming the RCFE numbers apply.

### 5.3 Staffing & night supervision (ARF requirements)

| Situation                             | Night supervision required                                                              |
|---------------------------------------|-----------------------------------------------------------------------------------------|
| 15 or fewer residents                 | At least one staff person on-call and on the premises                                   |
| 16–100 residents                      | At least one staff on-duty and awake, plus another on-call reachable within 30 minutes  |
| 7+ residents needing TOTAL assistance | At least one awake on-duty staff, plus one additional awake staff per 14 such residents |

For residents who need total assistance with daily living during the day, plan at least one direct-care staff per three such residents. A brain-injury home's acuity may require richer staffing than these minimums — staff to your residents' needs.

## 5.4 Background checks — Live Scan (HSC § 1522)

1. **Live Scan everyone required.** The licensee, administrator, every employee with client contact, and any adult living in the home must be fingerprinted via Live Scan, submitted to the California DOJ and FBI.
2. **Budget the fees.** The CCLD fingerprint fees for an ARF are \$42 (DOJ) + \$17 (FBI) = \$59, plus the Live Scan site's rolling fee (varies by location).
3. **Know the exemption path.** A prior conviction doesn't automatically bar you — HSC § 1522 has a criminal-record exemption process (some offenses are non-exemptible). CDSS reviews and decides.
4. **Document everything.** Keep the clearances in each file; the analyst will verify them before licensure.

*What this means for you — start clearances first, hire second.* A Live Scan can take days to weeks to come back, and no one with client contact may work before their clearance (or an approved exemption) is on file. So run YOUR clearance and your administrator's the moment you form the entity, and require a completed Live Scan as a condition of every job offer. Budget the \$59 CCLD fee plus the site's rolling fee per person, and never let an uncleared person work a shift with residents — it's one of the fastest ways to a citation.

## 5.5 Build the file every staff member needs

CCLD reviews personnel files at licensing and on later visits. Set up one folder per employee before you open, and keep each of these in it:

- **Cleared Live Scan:** the DOJ/FBI criminal-record clearance (or an approved HSC § 1522 exemption) dated before their first shift with residents.
- **Health screening:** a health screening/TB clearance as required for direct-care staff — confirm the current form with CCLD.
- **Training records:** dated proof of the required topics (Part 5.2), current first aid/CPR cards, and your own brain-injury competency training.
- **Job description & orientation:** a signed job description and documentation that the person was oriented to residents' rights, your emergency plan, and reporting duties.

*You are all mandated reporters.* Every staff member is a mandated reporter of suspected abuse, neglect, or exploitation. Train it on day one, post the Ombudsman (1-800-231-4024) and APS (1-833-401-0832) numbers where residents and staff can see them, and document that each employee knows how and when to report.

## PART 6

# Get Your CDSS License — Step by Step

**In short:** Attend the required orientation *FIRST*, then submit the LIC 200 with your Plan of Operation, clearances, and financials; get the fire clearance; pass the pre-licensing visit. CDSS targets 90–120 days.

## 6.1 Attend orientation (Component I) — before you apply

California requires you to complete a category-specific orientation before you submit an application. An online orientation is accepted. This is Component I of a three-part process: Component II is a one-on-one review meeting with your assigned analyst after you apply, and Component III is a category-specific requirements review before licensure (your administrator attends II and III if different from you).

## 6.2 Assemble & submit the application

- ☐ LIC 200 — Application for a Community Care Facility License (with LIC 281 instructions)
- ☐ Your Plan of Operation (how the home runs day to day) and program/admission policies
- ☐ LIC 500 Personnel Report and LIC 9282 Infection Control Plan
- ☐ Criminal-record (Live Scan) clearances for all required persons
- ☐ Fire clearance from your local fire authority
- ☐ Financial verification: at least 3 months of operating reserve in the applicant's name; an operating budget (budget at least ~\$200/person/month for food); credit history
- ☐ A 7-day supply of non-perishable food on hand at the pre-licensing inspection

## 6.3 Pay, inspect & license

- 1. Pay the fees.** The initial application fee and the annual fee are set by capacity (Health & Safety Code § 1523.1). Confirm the current dollar amounts on the CCLD fee-payment site or with the Adult and Senior Care Program — don't rely on a guessed figure or a number quoted for an RCFE.
- 2. Complete the analyst review (Component II).** Your assigned Licensing Program Analyst reviews your application and meets with you.
- 3. Pass the pre-licensing inspection.** CCLD inspects the home against Title 22 before issuing the license.
- 4. Receive your ARF license.** Then operate to the standards — CCLD inspects over time and investigates complaints.

## 6.4 Timeline

CDSS targets 90–120 days from a complete application, but the fire clearance, background clearances, over-concentration checks, and any construction can extend it. Start the orientation, the administrator course, and the Live Scans in parallel so your submission is complete.

**Ask before you file.** Call the ASCP Centralized Applications Bureau at (833) 827-6084 to confirm the current forms, fees, orientation schedule, and required attachments — a complete first submission saves weeks.

## PART 7

# The Resident's Journey — Referral to Discharge

**In short:** A resident is referred (by a hospital/rehab discharge planner, a county agency, or a family); you confirm the fit, admit under an admission agreement, deliver and document the needs & services plan, and plan for the least-restrictive setting. Here is the whole journey, stage by stage.

### Stage 1 — Referral comes in

A referral comes from a hospital or rehab (e.g., Rancho Los Amigos) discharge planner, a county behavioral-health or social-services worker, a Regional Center (for the DD-eligible), or a family (Part 10). Someone asks whether you have a bed for a specific person.

#### What you do:

- Take the basics (age — confirm 18–59 for an ARF, diagnosis, funding source, target move date). Ask for the person's assessment and any service plan. Tell them your license, capacity, and current openings, and that you're non-medical.

### Stage 2 — Pre-admission appraisal (can you safely serve this person?)

Before you say yes, confirm the resident fits an ARF, your capacity, and your abilities. An ARF is non-medical — if the person has a “restricted health condition,” you may serve it only with appropriately skilled staff and a plan/waiver; if they're an elopement risk, remember you can't lock the perimeter, so be honest about whether supervision and design can keep them safe.

#### What to review (the LIC 603 appraisal):

- Diagnoses and behaviors (aggression, elopement/wandering, swallowing/choking risk); mobility and transfer needs; medications and self-administration ability; and whether your staffing and non-secured setting can meet all of it safely. When in doubt, ask the referring clinician.

### Stage 3 — Admission & the agreement

On move-in, the resident (or their representative) signs an admission agreement (California uses standard admission-agreement requirements) covering services, rights, fees, who pays what, and how residency can end. Give a rights orientation and open a resident record.

#### The resident record must include:

- Health information and diagnoses; a current medication list with allergies; advance directives; physician and case-manager contacts; emergency contacts and the representative; and the signed agreement and consents.

### Stage 4 — Build the needs & services plan (the roadmap)

Develop and follow the resident's needs and services plan — the day-to-day plan for THIS person. For a brain-injury resident it typically covers cognitive-stimulation activities, wander- and personal-safety (without a secured perimeter), help with the activities of daily living, assistance with self-administered medication, nutrition and any special diet, and behavior supports. Coordinate with the physician and any case manager, and have the resident/representative sign off.

### Stage 5 — Daily life & care

Deliver the plan: structured daily routines, ADL support, cognitive and social activities, community outings as appropriate, meals, and safe medication assistance — all mapped to the resident's goals. Maintain your required staffing and night supervision, and document progress notes.

### Stage 6 — Reviews, incidents & rights

Review and update the plan as needs change. Run your quality program with incident reports (and file the required CCLD reports for serious incidents); protect resident rights; and report any suspected abuse, neglect, or exploitation immediately. Everything is documented and followed up.

### Stage 7 — Discharge & transition

When a resident's needs change or goals are met, coordinate a safe transition with the case manager and family — to a less-restrictive setting, a higher level of care (a medical/nursing setting if they exceed non-medical care), or the community — following the admission agreement's termination terms and giving proper written notice. Send the receiving provider the records they need, and document the transition.

***What this means for you — plan discharge on day one.** Because an ARF is non-medical, some residents will eventually exceed what you can safely provide — a swallowing decline, a wound that needs skilled nursing, or behavior your non-secured setting can't keep safe. That is not a failure; it's the least-restrictive-setting principle working as intended. Write your transition process into the admission agreement up front (notice periods, who arranges the next placement, how records transfer), and start the conversation with the case manager and family EARLY rather than in a crisis. A calm, well-documented discharge protects the resident, keeps your relationship with the referring hospital intact, and keeps you clear of a wrongful-eviction complaint.*

## **7.1 The documents that follow the resident the whole way**

Each stage above produces a record CCLD expects to see. Keeping them current is most of what a clean compliance file looks like:

- **Pre-admission appraisal (LIC 603):** documents that you assessed fit before admitting — diagnoses, behaviors, mobility, medications, and elopement risk against your non-secured setting.
- **Admission agreement & rights orientation:** the signed agreement covering services, fees, who pays what, and termination — plus proof you gave the rights orientation.
- **Needs & services plan:** the living roadmap for THIS resident, signed by them or their representative and updated as needs change.
- **Medication and progress records:** a current medication list with allergies, the medication-assistance log, and dated progress notes.
- **Incident reports:** your internal reports plus the required CCLD reports for serious incidents — filed on time and followed up.

## PART 8

## How You Get Paid — SSI/SSP & Private Pay, Step by Step

**In short:** Most brain-injury ARF residents are paid through SSI/SSP board & care at the fixed NMOHC rate — not a Medicaid per-diem. Regional Center funding applies only to the few whose disability began before 18. Build your model around SSI/SSP + private pay.

Summary of the sources:

| Source          | What it is                                           | Key fact                                                                                  |
|-----------------|------------------------------------------------------|-------------------------------------------------------------------------------------------|
| SSI/SSP (NMOHC) | Federal SSI + California State Supplementary Payment | The main payer: a fixed monthly board-and-care rate for residents in a licensed ARF       |
| Private pay     | Family/self-pay                                      | Fills the gap above the SSI/SSP rate; covers non-SSI residents                            |
| Regional Center | Lanterman Act (21 Regional Centers)                  | ONLY if the disability began before age 18 — most adult-onset TBI residents don't qualify |
| Medi-Cal / VA   | Health coverage; VA benefits                         | Medi-Cal may pay a resident's MEDICAL care (not room & board); VA A&A may help a veteran  |

### 8.1 The SSI/SSP board-and-care rate (your main revenue)

California pays a State Supplementary Payment on top of federal SSI for people who live in a licensed non-medical residential facility — the Non-Medical Out-of-Home Care (NMOHC) rate. Effective January 1, 2026, the total NMOHC payment is \$1,626.07/month (federal SSI \$994.00 + California SSP \$632.07), out of which the resident keeps a personal-and-incidental-needs allowance (at least \$182) and the rest goes toward their board and care. Verify the exact current figure on the California DHCS SSI/SSP payment-standards table before you quote it — the amount changes with the annual cost-of-living adjustment.

***What this means for your business plan.** At roughly \$1,626/month per SSI/SSP resident, a small six-bed home's SSI/SSP revenue is modest — most successful brain-injury ARFs blend SSI/SSP residents with private-pay residents (families paying a higher monthly rate for a specialized brain-injury program) to make the economics work. Model both, and set a clear private-pay rate.*

### 8.2 Regional Center — only for disability before 18

California's 21 Regional Centers fund residential and day services for people with developmental disabilities — but a developmental disability must have originated before the person's 18th birthday (Welfare & Institutions Code § 4512). An adult who acquired a brain injury later generally does NOT qualify. If you will serve residents whose qualifying disability DID begin before 18, you can apply to become a “vendored” provider of the Regional Center in your area (vendorization), which pays a negotiated rate — but do not assume adult-onset TBI residents are eligible. Confirm each resident's status through the Regional Center.

### 8.3 Private pay, Medi-Cal & veterans

- **Private pay:** families paying for a specialized brain-injury program are a core part of most ARFs' revenue — set a clear monthly rate and a written admission/financial agreement, reviewed by counsel.
- **Medi-Cal:** most brain-injury residents will have Medi-Cal for their MEDICAL care (doctor visits, therapies, medications), but Medi-Cal does NOT pay ARF room and board — keep the two straight.
- **Veterans:** VA Aid & Attendance (an add-on to the VA pension) can help fund a veteran's care — 2025–2026 maximums are up to about \$2,424/month for a single veteran and about \$1,558/month for a surviving spouse (verify on VA.gov). The VA Polytrauma/TBI System of Care is also a resource.

### 8.4 Getting set up to be paid

1. License the ARF first (Part 6).

2. **Help residents establish SSI/SSP** — work with the resident/representative and the county so the resident's SSI is coded for the out-of-home-care (NMOHC) rate once they live in your licensed facility.
3. **Set your private-pay rate and agreement** — in writing, reviewed by counsel.
4. **If serving before-18 disability residents, vendorize** with your Regional Center; and get your Type 2 NPI (Part 3.7) for any billing or accreditation.

## 8.5 Build the revenue model — a worked example

Numbers make the strategy concrete. Take a six-bed home and note how the payer mix changes the picture (illustrative — use your own market's private-pay rate and verify the current NMOHC figure):

- **All six on SSI/SSP:** six residents at about \$1,626/month is roughly \$9,750/month gross — before rent, payroll, food, and insurance. For most homes that alone does not clear the cost of 24-hour staffing.
- **A blended mix:** three SSI/SSP residents plus three private-pay families at a specialized brain-injury rate changes the math substantially — the private-pay rate is what you set and defend on the quality of your program (structured cognitive activities, low staff-to-resident ratio, brain-injury-trained staff).
- **Add-ons, kept straight:** a veteran's VA Aid & Attendance may supplement that resident's private share, and Medi-Cal covers the resident's MEDICAL care — but never room and board. Don't book Medi-Cal as facility revenue.

***What this means for you — set and defend a private-pay rate.** The single biggest lever on whether the business works is your private-pay rate and how many private-pay beds you fill. Set it in a written admission/financial agreement reviewed by counsel, tie it to the specialized brain-injury services you actually deliver, and revisit it each year as the SSI/SSP rate updates with the cost-of-living adjustment. Confirm the current NMOHC amount on the DHCS payment-standards table before you quote any resident or lender.*

## PART 9

## What It Costs & How Long It Takes

**In short:** Budget the home, the 35-hour administrator course, staff training, Live Scans, the CDSS fees, the \$800 franchise tax, and three months of reserve — and consider accreditation. Plan ~90–120 days to license.

### 9.1 Startup line items (typical — verify)

| Item                                     | What's in it                                                               | Ballpark                               |
|------------------------------------------|----------------------------------------------------------------------------|----------------------------------------|
| The home                                 | Lease deposit + first month (or purchase); accessibility mods; furnishings | Varies by market/size                  |
| Operating reserve                        | At least 3 months of operating costs, in the applicant's name (required)   | Several thousand \$+                   |
| Administrator ICTP                       | 35-hour course + \$100 exam + \$100 certificate                            | ~\$400–700 all in                      |
| Staff training                           | First aid/CPR and the required topics per staff                            | Per person                             |
| Live Scan                                | \$59 (DOJ \$42 + FBI \$17) + the site rolling fee, per person              | Per person                             |
| Insurance                                | GL + professional + abuse + property + auto + workers' comp                | Get quotes                             |
| CDSS fees                                | Initial application + annual fee by capacity (HSC § 1523.1)                | Confirm current amounts                |
| Business setup                           | LLC-1 \$70 + \$800 franchise tax + LLC-12 \$20, EIN/NPI free, legal        | ~\$900 + legal                         |
| Policies                                 | P&P Manual + Plan of Operation + Forms                                     | One-time                               |
| Accreditation (optional but recommended) | CARF or Joint Commission (see 9.2)                                         | Low-to-mid five figures over the cycle |

### 9.2 If you pursue accreditation (recommended for brain injury)

- **CARF:** its Brain Injury specialty accredits residential brain-injury programs under the Medical Rehabilitation standards, and requires you to be operating and serving clients before survey. Commonly cited fees are a ~\$995 application fee plus a variable surveyor-day survey fee — request a current quote. (888) 281-6531, [carf.org](http://carf.org).
- **The Joint Commission:** offers Behavioral Health Care & Human Services accreditation; fees are quote-based on your services and census, on a 3-year cycle. Request a quote: (630) 792-5000, [jointcommission.org](http://jointcommission.org).
- **It's staged:** both require you to be operating before survey, then an initial on-site survey, then a decision. Plan roughly 12–18 months. The VA's brain-injury programs are CARF-accredited, which tells you what payers value.

### 9.3 Timeline

|                    |                                                           |                                                                              |
|--------------------|-----------------------------------------------------------|------------------------------------------------------------------------------|
| <b>Early</b>       | <b>Orientation &amp; administrator</b>                    | Attend the CCLD orientation; start the 35-hour ICTP; begin Live Scans        |
| <b>Weeks</b>       | <b>Business &amp; home ready</b>                          | Entity (\$800 tax), insurance, home to Title 22 specs, zoning + reserve      |
| <b>Weeks</b>       | <b>Plan &amp; staff ready</b>                             | Plan of Operation, staff topics/first aid, forms, food supply                |
| <b>90–120 days</b> | <b>LIC 200 → fire clearance → pre-licensing → license</b> | Application → analyst review → inspection → ARF license                      |
| <b>Then</b>        | <b>Set up funding</b>                                     | SSI/SSP (NMOHC) coding + private-pay agreements; Regional Center if eligible |
| <b>Ongoing</b>     | <b>Referrals &amp; full census</b>                        | Rancho/hospital discharge planners + county agencies + families              |

## PART 10

## Getting Your First Residents

**In short:** Residents reach you through hospital and rehab discharge planners — Rancho Los Amigos and the Level 1 trauma/rehab centers are the brain-injury pipeline — plus county agencies and families. Build those relationships before you open.

### 10.1 The referral sources

- **Hospital & rehab discharge planners:** the biggest pipeline for brain injury — the trauma centers and neuro-rehab hospitals below discharge patients who need a residential home. Build the relationship with their case-management/social-work teams; leave a one-pager and ask how to get on their placement list.
- **County behavioral health & social services:** for residents with co-occurring mental-health needs and for SSI/placement help, county agencies and public guardians place adults into licensed ARFs — get known to them.
- **Brain Injury Association of California (BIACAL) & support networks:** BIACAL connects survivors and families to services (biacal.org) — a two-way referral relationship. California also has community-based brain-injury reentry programs; ask BIACAL and your regional rehab hospital which serve your area.
- **Regional Centers (for the eligible few):** if you vendorize, the Regional Center places its clients — but remember only before-18-onset residents qualify.

### 10.2 Trauma & neuro-rehab centers (verify before relying)

| Metro                | Program (referral source)                                             | Contact (verify)       |
|----------------------|-----------------------------------------------------------------------|------------------------|
| Los Angeles (Downey) | Rancho Los Amigos National Rehabilitation Center — flagship TBI rehab | (562) 401-7022         |
| Los Angeles          | LAC+USC Medical Center (Level 1 trauma); Cedars-Sinai                 | LAC+USC (323) 409-1000 |
| San Jose             | Santa Clara Valley Medical Center — noted TBI model-system rehab      | (408) 885-5000         |
| Pomona               | Casa Colina Hospital & Centers for Healthcare (rehab)                 | (909) 596-7733         |
| San Diego            | Sharp Rehabilitation Center; UC San Diego Health (Level 1)            | Sharp (858) 939-3299   |
| Sacramento           | UC Davis Medical Center (Level 1 trauma + rehab)                      | (916) 734-2011         |
| San Francisco        | Zuckerberg SF General (Level 1); UCSF                                 | ZSFG (628) 206-8000    |

#### Ask each referral source:

- How do you decide where to place brain-injury patients (age 18–59) who need a residential home? What do you need from me to be on your placement list (license, capability, availability)? Who is the discharge planner / social worker I should stay in touch with?

## PART 11

## Who to Call & Exactly What to Ask — Every Verify, in One Place

**In short:** Your master contact + verification hub. Everywhere the guide says “verify,” the who and the what-to-ask are collected here.

### Licensing, the administrator & the building

| Who                                       | Contact                                             | What to verify / ask                                                                      |
|-------------------------------------------|-----------------------------------------------------|-------------------------------------------------------------------------------------------|
| CDSS/CCLD — ASCP Centralized Applications | (833) 827-6084 · CCLASCPCAB@dss.ca.gov              | The orientation; the LIC 200; current fees; required attachments; the pre-licensing visit |
| CDSS Adult Care Licensing                 | (916) 657-2592 · cdss.ca.gov (Adult Care Licensing) | Program questions; the Plan of Operation; restricted health conditions                    |
| Administrator Certification (ICTP vendor) | cdss.ca.gov (approved vendors)                      | The 35-hour ARF course; the state exam; recertification CE                                |
| Local fire authority / State Fire Marshal | your jurisdiction · osfm.fire.ca.gov                | The fire clearance; occupancy; the secured-perimeter limits                               |
| City/County Planning                      | your jurisdiction                                   | Residential-zoning protection for 6-or-fewer — in writing                                 |
| Live Scan (CA DOJ)                        | oag.ca.gov (Live Scan)                              | Fingerprint clearances; the \$59 CCLD fee + rolling fee; the exemption process            |

### Business, insurance & billing IDs

| Who                                     | Contact                                        | What to verify / ask                                                |
|-----------------------------------------|------------------------------------------------|---------------------------------------------------------------------|
| California Secretary of State           | 916-657-5448 · bizfileOnline.sos.ca.gov        | Forming your LLC (LLC-1, \$70); the LLC-12; the \$800 franchise tax |
| Franchise Tax Board                     | ftb.ca.gov                                     | The \$800 minimum tax (no first-year waiver now) and filing         |
| IRS                                     | irs.gov (EIN Assistant)                        | Get your EIN (free)                                                 |
| NPPEs / CMS                             | nppes.cms.hhs.gov                              | Your Type 2 NPI (for vendorization / accreditation)                 |
| Insurance agent (Trusted Choice / NIAC) | trustedchoice.com; insurancefor nonprofits.org | Coverages & limits; abuse & molestation; workers' comp              |

### Funding & referrals

| Who                                       | Contact                                  | What to verify / ask                                                |
|-------------------------------------------|------------------------------------------|---------------------------------------------------------------------|
| Social Security Administration (SSI)      | 1-800-772-1213 · ssa.gov                 | Establishing a resident's SSI and the out-of-home-care coding       |
| California DHCS — SSI/SSP standards       | dhcs.ca.gov (SSI/SSP payment standards)  | The current NMOHC monthly rate & personal-needs allowance           |
| Your Regional Center (if before-18 onset) | dds.ca.gov (find your Regional Center)   | Eligibility; vendorization; the negotiated rate                     |
| County Behavioral Health / APS            | your county · APS 1-833-401-0832         | Placements; public guardian; abuse reporting                        |
| Accreditor — CARF / TJC                   | CARF (888) 281-6531 · TJC (630) 792-5000 | Cost quote; the operating-first requirement; Brain Injury specialty |

### Advocacy, the state BI org & referral partners

| Who                                       | Contact                                     | Role                                                                 |
|-------------------------------------------|---------------------------------------------|----------------------------------------------------------------------|
| Long-Term Care / Community Care Ombudsman | 1-800-231-4024 (CRISIS line) · aging.ca.gov | Independent resident-rights advocate — post the number for residents |
| Adult Protective Services (county)        | 1-833-401-0832                              | Mandatory abuse/neglect/exploitation reporting                       |
| Brain Injury Association of California    | biacal.org                                  | Survivor/family support & resources; two-way referrals               |
| Rancho Los Amigos Nat'l Rehab Center      | (562) 401-7022 · rancho.org                 | The flagship CA brain-injury rehab — your key discharge relationship |
| CalABLE / benefits planning               | calable.ca.gov                              | Helping residents keep benefits while                                |

## You can do this

Opening a California brain-injury adult residential facility is a real build — a licensed ARF to Title 22 specs, a certified administrator, trained staff, and vulnerable residents — but it's a defined path. The two things that make California different are the funding (SSI/SSP board-and-care plus private pay, since most adult-onset TBI residents aren't Regional Center-eligible) and the secured-perimeter limit — plan for both from the start. Take it one rung at a time: attend orientation and certify your administrator, form the business and fund the reserve, prepare the home and team, get licensed by CDSS, set up SSI/SSP and private-pay funding, and earn referrals from Rancho and the trauma centers. Verify each rate and contact as you go, and lean on the people in Part 11.

**Reminder.** Prepared for PolicyReady.org and grounded in Title 22, Division 6, Chapter 6, the California Community Care Facilities Act (HSC § 1500 et seq.), and current SSI/SSP information as of 2026. This is general information, not legal, tax, or billing advice. Requirements, fees, rates, and contacts change — verify each with CCLD, your fire authority, DHCS/SSA, and qualified counsel before you rely on it. Confirm the current NMOHC rate and the CDSS fee amounts before you rely on them.